



OFFICIAL

Metabolic Screening Procedure

1. Scope

Site	Operational Area	Applicable to
Armadale Kalamunda Group	Mental Health	Medical, Nursing and Allied Health

2. Purpose

All consumers of Armadale Mental Health Service (AMHS) will have their physical health assessed and regularly monitored to ensure current and ongoing needs are identified and managed in a timely and effective manner to promote optimal physical health and wellbeing.

3. Policy statement

Metabolic screening will occur for all consumers admitted to and/or activated by the Armadale Mental Health Service. The Metabolic Screening Assessment Tool (MSAT) should be used for **ALL** community consumers and consumers admitted to the Inpatient Units who are prescribed psychotropic medication.

4. Background

Metabolic syndrome is a recognised cluster of features predictive of both cardiovascular disease and type 2 diabetes. A person must have

Central obesity **plus any of the following two**;

- A high triglyceride level
- A low HDL cholesterol level
- High blood pressure
- High fasting blood sugar

Metabolic syndrome is related to several factors including, but not limited to –

- Gender
- Medication use
- Lifestyle factors
- Psychosocial factors
- Comorbid physical illness

People with mental health problems are likely to have a higher rate of these risk factors, placing them at a higher risk of chronic disease co-morbidities. Mental Health consumers, particularly those with diagnoses of schizophrenia, bipolar disorder and major depressive disorders have up

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to four times greater risk of developing Metabolic Syndrome than the general population because of lifestyle factors and the side effects of medication treatment regimens.

5. Procedure

5.1 Community Mental Health Services

- In the community setting, it is the responsibility of the Treating Team to ensure that the screening observations are conducted at the recommended intervals in community.
- The Care Coordinator will ensure initial observations are completed as per the metabolic screening form; if the care co-ordinator is from a non –nursing background, the responsible medical practitioner will conduct the blood pressure assessment. All consumers will have baseline investigations conducted as per the metabolic screening assessment form by the Medical Practitioner. The care co-ordinator, if not already available, will contact the consumer's GP for: -
 - List of current physical health problems
 - List of current medication(s) for ALL conditions
 - Copies of any recent blood tests, ECG or other investigations.
- Baseline observations are to be commenced at the first assessment in the community by the care coordinator or delegate. If care coordinator or delegate is non-nursing, blood pressure is to be completed by medical practitioner.
- Repeat observations are to be conducted every three months, by the Care Coordinator or delegate as per (Appendix 1). Some medications cause rapid weight gain; therefore, the consumer's weight gain will need to be monitored as per medical practitioner instruction.
- Metabolic Screening results are to be considered and discussed at consumers three monthly Clinical Review Meetings and form part of the Treatment Support Discharge (TSD) plan for managing physical health.
- The MSAT should be filed in Digital Medical Record (DMR) with accompanying Medication Profile Form.

Refusals

- Consumers refusing the assessment will have their decision documented in their medical file and on the screening form by the Care Coordinator.
- Communication with the consumer's GP will be maintained regarding the assessment and reattempts will occur at each contact with the consumer and documented.

Data recording

- The results of the MSAT will be entered into the appropriate boxes contained on the metabolic screening form by the care coordinator or medical staff (for inpatients) as per Appendix 1, initialled and dated by the identified clinician.
- An authorised signed entry is to be completed in the health record progress notes on each occasion that metabolic screening has occurred by the clinician conducting the assessment.

Abnormal results

- If Metabolic Syndrome is present or any individual abnormality identified, this will be documented in the health record, treatment support discharge plan updated by the care

coordinator and medical practitioner conducting the assessment and appropriate intervention implemented.

- In case of abnormal MST results, further investigations and assessments or repeat screening may be conducted as per the instructions of the registered medical practitioner and the management plan is to be updated accordingly. Clear management plans must be formulated in the Treatment Support & Discharge Plan (TDSP), updated and reviewed regularly to address the abnormalities detected through Metabolic Screening Tool. This should be led by the responsible medical practitioner, and they will work with the consumer, care-co-ordinator, carers consumer's GP or a general physician to optimise Metabolic Syndrome parameters.

5.2 Inpatient Mental Health Services

- In the inpatient unit, it is the responsibility of the Resident Medical Officers (RMO's) of the clinical team to complete these observations and to fill the MST form.
- MSAT should be completed in ALL patients on psychotropic medication.
- Abnormal results are to be included in the clinical handover.
- All consumers to have a comprehensive Mental Health Physical Assessment Examination on admission or within 12 hours of admission. CRHTT consumers transferred from in-patient will have their existing Mental Health Physical Assessment Examination copy sent to CRHTT. Consumers admitted to CRHTT will be encouraged to have their comprehensive physical assessment completed by their GP during their admission to service, identified GP will be sent a GP letter on admission. All CRHTT consumers will have their baseline vital observations completed on admission, repeat observations are completed as identified.
- Abnormal results and refusal to engage in metabolic screening to be escalated to the treating consultant psychiatrist, TSD plans to be updated with identified intervention and monitored through ward round reviews.
- Consumers discharged from inpatient mental health units who have had metabolic screening, will have the results included in the clinical handover to care coordinator and in the discharge summary to GP's.

6. Legislative context

The following documents are required to give effect to this policy:

- [WA Mental Health Act \(WA\) 2014](#)

7. Supporting information

The following documents support the implementation of this policy:

- EMHS
 - [EMHS Care Coordination in Mental Health Policy](#)
- AKG
 - [Clozapine Prescribing Administering and Monitoring Policy](#)
 - [After Hours Clozapine Acquisition Procedure](#)
 - [Assessment Requirements for Mental Health Consumers Procedure](#)
 - [Metabolic Screening Assessment AKMR 169.5](#)

8. Compliance, monitoring and evaluation

Compliance against this policy will be evaluated with audit and evaluation of compliance when undertaking a related clinical incident investigation.

9. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- [Standard 2 - Criterion 2.11](#)
- [Standard 5 – Action 5.3](#)
- [Standard 5 – Action 5.10](#)
- [Chief Psychiatrists Clinical Care Standards](#)

10. References

1. [Clinical Guidelines for the Physical Care of Mental Health Consumers, The University of Western Australia \(2010\).](#)

11. Glossary

The following definitions are relevant to this policy.

Term	Definition
Metabolic Screen	An evidence-based process for health professionals/services to regularly review consumers' physical health status via assessment, monitoring and input.
Assessment Tool	An evidence-based tool is utilised to aid clinicians in the screening and monitoring of metabolic syndrome for consumers prescribed antipsychotic, anti-depressant and mood stabiliser medications. The screening assessment tool provides a single point of reference for data to be recorded and located. The screening assessment tool provides an historical view of a consumer's physical health observations over a twenty-four-month period.

12. Policy owner

Service Director Mental Health

Enquiries relating to this policy may be directed to:

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13. Review

This policy will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
3	13 November 2025	13 November 2028	Full review with only minor amendments

14. Authorisation

This policy has been authorised and issued by:

Authorised by	Karen Elliott - Service Director Mental Health.
Authorisation date	12 11 2025
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